Health and Safety Policy

Practice	Kirkcaldy Dental & Implant Clinic, 23–25 Tolbooth Street, Kirkcaldy KY1 1RW
PSM section	Health and Safety – General › Health and safety legislation, risk management, premises, electricity, DSE, manual handling, mental health, RIDDOR, medical devices, pressure vessels, lasers
Policy lead	Health & Safety Lead [Name]
Version	1.0 (draft for adoption)
Adopted	[date]
Next review	[date – 12 months after adoption]

1. Purpose

This is the written health and safety policy of Kirkcaldy Dental & Implant Clinic. It sets out:

- our statement of intent to provide a safe and healthy workplace;
- who does what, when and how;
- the person responsible for the policy (the employer) and the person responsible to the employer for putting it into practice.

A written policy is a legal requirement once five or more people work in the practice. The PSM strongly advises all practices to have one, so we have one from the day we open.

2. Scope

This policy covers everyone at the practice: employees, self-employed associates and hygienists/therapists, trainees, locums, agency staff, contractors, patients and visitors. It covers the whole premises at 23–25 Tolbooth Street and all work activities, NHS and private (including implant surgery).

Infection prevention and control and radiation protection have their own policies (see section 8). Fire, COSHH, first aid, latex, waste and PPE also have separate policies in this set; this document is the umbrella policy that links them.

3. Legislation and guidance

As cited in the PSM:

- Health and Safety at Work etc Act 1974
- Management of Health and Safety at Work Regulations 1999 and (Amendment) Regulations 2006
- Health and Safety Information for Employees Regulations 1989
- Workplace (Health, Safety and Welfare) Regulations 1992
- Electricity at Work Regulations 1989; HSE HSR25 guidance (2015); HSE *Maintaining portable electrical equipment in offices and other low risk environments* (2013)
- Health and Safety (Display Screen Equipment) Regulations 1992, as amended by the Health and Safety (Miscellaneous Amendments) Regulations 2002

- Manual Handling Operations Regulations 1992; HSE INDG143; HSE *Moving and handling in health and social care* (2024)
- Provision and Use of Work Equipment Regulations 1998 (PUWER)
- Pressure Systems Safety Regulations 2000 (PSSR); HSE *Pressure Systems: a brief guide to safety*; HSE *Written schemes of examination*
- SHTM 01-05 (Health Facilities Scotland, 2024) – decontamination in a Local Decontamination Unit
- Control of Artificial Optical Radiation at Work Regulations 2010; Health and Safety (Safety Signs and Signals) Regulations 1996; BS EN 60825-1; MHRA laser guidance (2015)
- Medical Devices Regulations 2002 (UK MDR 2002)
- Reporting of Injuries, Diseases and Dangerous Occurrences Regulations 2013 (RIDDOR); HSE Accident Book BI510
- HSE and Public Health Scotland – Healthy Working Lives guidance; FDI World Dental Federation mental health toolkit

Health and safety at work law is reserved to the UK Government and enforced by the Health and Safety Executive (HSE), which can impose penalties for non-compliance.

PSM pages:

- https://www.psm.sdcep.org.uk/topics/health-and-safety-general/
- https://www.psm.sdcep.org.uk/topics/health-and-safety-general/health-and-safety-legislation/general-requirements-for-compliance-by-the-employer/
- https://www.psm.sdcep.org.uk/topics/health-and-safety-general/health-and-safety-legislation/duties-of-employees-and-the-general-public/
- https://www.psm.sdcep.org.uk/topics/health-and-safety-general/health-and-safety-risk-management/five-steps-to-a-health-and-safety-risk-assessment/
- https://www.psm.sdcep.org.uk/topics/health-and-safety-general/premises/
- https://www.psm.sdcep.org.uk/topics/health-and-safety-general/electricity-at-work/
- https://www.psm.sdcep.org.uk/topics/health-and-safety-general/display-screen-equipment/
- https://www.psm.sdcep.org.uk/topics/health-and-safety-general/manual-handling/
- https://www.psm.sdcep.org.uk/topics/health-and-safety-general/mental-health-and-wellbeing-at-work/
- https://www.psm.sdcep.org.uk/topics/health-and-safety-general/pressure-vessels/
- https://www.psm.sdcep.org.uk/topics/health-and-safety-general/lasers/
- https://www.psm.sdcep.org.uk/topics/health-and-safety-general/medical-devices/
- https://www.psm.sdcep.org.uk/topics/health-and-safety-general/riddor-reporting-of-injuries-diseases-and-dangerous-occurrences-regulations-2013/

4. Responsibilities

Statement of intent. Kirkcaldy Dental & Implant Clinic will, so far as is reasonably practicable, provide a safe and healthy workplace for our team and make sure that patients, visitors and contractors are not put at risk by our work. We will assess risks, control them, give the team the information and training they need, and listen to their concerns.

Signed: _____ Practice Owner / Principal Dentist [Nas] Date: [date]

Role	Responsibility
Practice Owner / Principal Dentist [Nas] (the employer)	Legally responsible for health and safety. This stays true whoever is appointed as competent person. Signs this policy, provides resources, holds employer's liability and pressure vessel insurance.
Health & Safety Lead [Name]	The person responsible to the employer for putting this policy into practice. Keeps the risk assessments, registers and records up to date; runs the annual review; receives fault and incident reports.
Competent person (health and safety) [Name / external adviser – Company]	Advises on health and safety administration and compliance. May be the employer, an employee or an external adviser.
Practice Manager [Name]	Contracts (DSE, manual handling and PPE clauses), training records, contractor bookings (PAT, fixed wire, pressure vessel inspections).
Fire Safety Lead [Name]	See Policy 19 – Fire Safety Policy.
IPC Lead / Decontamination Lead [Name]	Day-to-day safe use of sterilizers and washer-disinfectors; PPE (see Policy 23 – Personal Protective Equipment (PPE) Policy).
Medical Emergencies Lead [Name]	Medical emergency arrangements (see Policy 34 – Medical Emergencies and Life Support Policy).
Laser Protection Adviser (LPA) [Name / Company] and Laser Protection Supervisor (LPS) [Name]	Only if a Class 3B or 4 laser is used (section 5.10).
Wellbeing / mental health wellness lead [Name]	Point of contact for wellbeing; keeps wellbeing on the practice meeting agenda.
All employees	Take reasonable care of their own health and safety and that of others affected by what they do or fail to do. Follow practice rules and training. Co-operate with the employer. Report faults, hazards and incidents. Failure to comply can lead to prosecution.
Self-employed associates and clinicians	Carry out their work so they do not put their own or others' health and safety at risk. Follow this policy and attend practice health and safety training.
Patients and visitors (including children over 8)	Must not intentionally interfere with or misuse anything provided for health and safety.

5. Policy / procedure

5.1 General employer arrangements

1. The Health & Safety Lead completes the health and safety risk assessment (5.2) **before** this policy is finalised, and reviews both together.
2. The policy and any changes are shared with the whole team at induction and at a practice meeting. Each person signs the sign-off table.

3. The HSE approved Health and Safety Law poster (2009 or 2018 version) is displayed at [location – e.g. staff room], or each team member is given the equivalent leaflet.
4. The current employer's liability insurance certificate is displayed at [location]. Insurer: [Company]; renewal date: [date].
5. Health and safety is a standing item on the practice meeting agenda. Staff can raise concerns and influence decisions there or at any time with the Health & Safety Lead.
6. Emergency procedures are in place for fire (Policy 19 – Fire Safety Policy), medical emergencies (Policy 34 – Medical Emergencies and Life Support Policy) and business continuity (Policy 38 – Business Continuity Plan).
7. First aid provision is set out in Policy 20 – First Aid Policy.
8. The Health & Safety Lead keeps up to date using the HSE and Healthy Working Lives websites and the NHS Fife Dental Practice Adviser.

5.2 Risk assessment – five steps

We use the PSM Health & Safety Risk Assessment template. Definitions: a **hazard** is anything with the potential to cause harm; **risk** is the likelihood that harm will occur; a **control** is a measure already in place; **extra controls** are measures still needed.

1. **Identify hazards.** Walk round the whole building, including the surgeries, LDU, X-ray areas, store, staff room, stairs, entrances and outside bin store. Look at physical, chemical, electrical, biological and work-related stress hazards.
2. **Assess the risks.** Decide who might be harmed and how: staff, patients, visitors, contractors, cleaners. Take account of pregnant workers and new mothers, young workers and temporary workers.
3. **Control the risks.** Evaluate the risks and decide on precautions, taking account of existing controls. Remove the hazard where possible; otherwise reduce it.
4. **Record your findings** and take the actions. Each action has a named owner and a target date.
5. **Review** the controls and update the assessment: at least annually [practice decision], and after any incident, new equipment, building change, new procedure or change in staff.

Individual risk assessments are done (and are a legal requirement) for:

- a team member who tells us she is pregnant or a new mother (as soon as we are told);
- any young worker (under 18) before they start;
- temporary workers.

All team members are involved in the risk assessment and its findings at a practice meeting.

5.3 Premises and welfare

Under the Workplace (Health, Safety and Welfare) Regulations 1992 the Health & Safety Lead ensures:

1. Adequate ventilation (fresh or purified air) without uncomfortable draughts.
2. Floors kept in good repair and non-slip where required.
3. A temperature of at least 16°C for sedentary staff.
4. Lighting adequate for each task.
5. Toilets and washbasins: 1 of each for 1–5 employees; 2 of each for 6–25 employees.
6. Premises kept clean, and waste removed promptly (Policy 22 – Waste Management Policy).
7. Drinking water for staff.
8. Changing facilities and a secure place to store personal belongings.

9. A rest and eating area away from clinical areas, with seats and a surface for food.
10. Premises kept in good working order. Staff report faults to the Health & Safety Lead, who records them in the maintenance log [location] and arranges repair.

5.4 Electricity at work and PAT

All electrical equipment must be built and maintained to prevent danger.

User checks (every user, before use).

1. Disconnect the equipment first.
2. Look for damage to the plug, flex and socket. Check cables are not trapped, kinked or trailing.
3. Do not use faulty equipment. Label it "Do not use", and report it to the Health & Safety Lead.
4. Do not dismantle plugs unless trained.
5. Record only if a fault is found (fault log).

Formal visual inspection by a trained competent person [Name]: dismantle plugs to check connections, fuses and casing; check extension leads; check no furniture sits on cables or blocks sockets. Frequency depends on how often the item is moved – for example kettle or vacuum cleaner 6–12 months; computer screen or phone 2–4 years. New appliances are assessed before first use (e.g. items bought from outwith the EU may have reversed polarity). All inspection results are recorded.

PAT testing by a competent person or electrical contractor (Energycert – same as Glenrothes). Equipment is labelled and a certificate and report is issued. Frequency is 1–5 years, set by our risk assessment (the law does not set a frequency). Not every item needs a PAT test. User checks continue between tests.

Fixed wiring. An electrical contractor (Energycert – same as Glenrothes) inspects and tests the mains installation and advises the retest interval. Date of last test: [date]; next due: [date]. We check whether our insurer requires this.

5.5 Pressure vessels (sterilizers and compressors)

1. Each sterilizer and the compressor has a **Written Scheme of Examination** drawn up by a Competent Person (Pressure Systems) – a chartered engineer as defined in PSSR 2000 – on installation or soon after. Small compressors under 250 bar-litres may be exempt from the written scheme, but maintenance and other requirements still apply.
2. Safety inspection and certification are arranged at the intervals in the written scheme – typically at least every 14 months for sterilizers and at least every 26 months for compressors. Inspector/insurer: [Company].
3. Inspection certificates are kept in the pressure systems file.
4. Maintenance follows the manufacturer's instructions. The register lists each vessel, serial number, written scheme date, last and next inspection.
5. Third party liability insurance for pressure vessels is a legal requirement. The Practice Owner checks that our policy covers them, as general surgery policies often exclude them.
6. Only trained staff operate sterilizers and the compressor (PUWER). Daily and weekly sterilizer tests are set out in Policy 26 – Decontamination of Reusable Instruments Policy and Procedures, following SHTM 01-05.

5.6 Display screen equipment (DSE)

1. The Practice Manager identifies DSE users: anyone who habitually uses a screen (including a laptop) daily, for continuous spells of an hour or more. Reception and Dentally users are likely to qualify.

2. Users and any workstation assessor receive the HSE *Working with display screen equipment* information and training on workstation set-up.
3. Each user's workstation is assessed using the PSM Display Screen Equipment Assessment template, with the user involved. Problems are corrected and the assessment is reviewed when the workstation or user changes.
4. Each workstation meets the minimum requirements in the Schedule to the Regulations (adjustable screen, separate tiltable keyboard, enough desk space, adjustable chair, adequate lighting).
5. Work is planned to allow breaks or changes of activity.
6. A user can ask for a DSE eye test, which the practice pays for. If spectacles are needed specifically for DSE work, the practice provides them [practice decision: budget].
7. Staff contracts include a clause on the correct use of DSE.

5.7 Manual handling

1. Avoid hazardous manual handling so far as is reasonably practicable – for example split stock deliveries, use a trolley, store heavy items at waist height.
2. Where a task cannot be avoided (deliveries, moving equipment, helping a patient into the dental chair), assess it using the Manual Handling Risk Assessment form (Healthy Working Lives) and put controls in place.
3. Staff do not lift patients. Patients with a physical disability are helped to the chair only as trained. If a hoist or transfer is needed, it is risk assessed in advance [practice decision: hoist available / refer]. See Policy 09 – Disability Equality and Reasonable Adjustments Policy.
4. All staff receive manual handling training (PSM Manual Handling Information template), including helping patients access the chair.
5. Manual handling duties are written into staff contracts.

5.8 Mental health and wellbeing at work

The employer must prevent work-related stress, depression and anxiety, and the risk assessment must include stress. Evidence of the risk assessment is required at Combined Practice Inspection. We follow the FDI three-step approach.

Determine. The Health & Safety Lead includes stress in the health and safety risk assessment, using the HSE stress risk assessment and the six domains: demands, control, support, relationships, role and change. The FDI assessment tool and Healthy Working Lives *Stress in the workplace* resource may be used.

Do.

1. A practice mental health wellness lead is named: [Name].
2. The Practice Owner considers signing the Mental Health Wellness in Dentistry Charter and following its Road Map [practice decision].
3. Staff are told about support: Dentists' Health Support Trust, ConfiDental, the National Wellbeing Hub, the Workforce Specialist Service (regulated professionals), NES dental wellbeing resources, NHS Inform Mind to Mind, and their GP. These are listed on the staff noticeboard.
4. The Working Minds Talking toolkit can be used to start conversations about workplace stress.

Discuss. Wellbeing is a standing item at practice meetings. Any team member can speak privately to the Practice Owner, Practice Manager or wellness lead. See also Policy 07 – Raising Concerns and Whistleblowing Policy.

5.9 Medical devices

1. We buy only dental materials, instruments and equipment that carry a UKCA or CE mark. Stock is checked on delivery.
2. Before using a dental laboratory, the Practice Manager confirms it is registered with the MHRA and keeps evidence in the supplier file. Laboratory: [Name], MHRA registration [number].
3. If we make devices in the practice (e.g. milled ceramics, retainers, implant restorations made chairside), the practice must register with the MHRA first. Status: [registered / not applicable].
4. Every custom-made device (from a lab or made in-house) is given to the patient with a patient statement. We use the GDC Statement of Manufacture template as the basis.
5. Impressions and appliances are disinfected before dispatch and the lab is told (Policy 31 – Dispatch to Laboratories, Equipment Suppliers and Pathology Services Policy).
6. **Reporting problems with a device.** If a device fails, is faulty, or causes or nearly causes harm, the clinician takes it out of use, keeps it with its packaging and batch details, records the event, and tells the Health & Safety Lead. The event is reported to Health Facilities Scotland (Incident Reporting and Investigation Centre) for NHS practices; the PSM directs purely private practices to the MHRA. As a mixed NHS/private practice we report to HFS and also to the MHRA if the device was used in private treatment [check current guidance – the PSM sets this out for X-ray equipment; confirm the route for other devices with NHS Fife]. The manufacturer is also told. The event goes through significant event analysis (Policy 37 – Risk Management and Significant Events Policy).
7. NES Turas has learning resources on medical devices regulation for staff.

5.10 Lasers (only if a Class 3B or Class 4 dental laser is used)

Current status: [the practice does / does not use a dental laser]. If not, this section does not apply until one is bought. Before any laser is used:

1. Appoint a Laser Protection Adviser (LPA) [Name / Company]. This is a requirement of practice inspection.
2. With the LPA: carry out the initial risk assessment; designate the laser controlled area; write local rules.
3. Appoint a Laser Protection Supervisor (LPS) [Name] with suitable training to oversee day-to-day safe use and compliance with the local rules.
4. Register use of the laser with Healthcare Improvement Scotland (or other regulator).
5. Display warning signs at the entrances to the room (Safety Signs and Signals Regulations; BS EN 60825-1).
6. Provide protective eyewear for staff and patients; gloves and coveralls for staff where appropriate.
7. Give documented training to everyone who works with the laser.
8. Obtain informed consent from patients for laser treatment (Policy 03 – Consent Policy).
9. Put fire prevention measures in place; follow a maintenance schedule with competent person inspections; run a quality assurance check system.
10. Provide health surveillance if skin or eyes are affected, and have a procedure for reporting possible over-exposure (referral to a doctor or occupational health).

5.11 Accidents, incidents and RIDDOR

1. **All staff** report every accident, incident or near miss as soon as possible to a senior member of the team (the Health & Safety Lead or the most senior person present).

2. Every accident is written in the practice's data protection compliant accident book (HSE BI510). Completed pages are removed and stored securely [location].
3. The Health & Safety Lead decides whether the event is reportable under RIDDOR. The "responsible person" reports: the employer for staff incidents; the person in control of the premises for self-employed people and members of the public.
4. **Reportable to HSE:**
   - death of any person from a work-related accident (not suicide);
   - specified injuries to workers: fractures (not fingers, thumbs or toes); amputations; permanent loss or reduction of sight; crush injuries to the head or torso causing brain or internal organ damage; serious burns (over 10% of the body, or damaging eyes, respiratory system or other vital organs); scalpings needing hospital treatment; unconsciousness from head injury or asphyxia; injuries from working in an enclosed space leading to hypothermia, heat illness, resuscitation or more than 24 hours in hospital;
   - over-seven-day incapacitation of a worker (more than seven consecutive days, not counting the day of the accident);
   - injury to a non-worker (patient or visitor) from a work activity where they are taken directly from the scene to hospital for treatment (not if taken purely as a precaution with no apparent injury);
   - occupational diseases likely caused or made worse by work, e.g. carpal tunnel syndrome, severe cramp of the hand or forearm, occupational dermatitis, hand-arm vibration syndrome, occupational asthma, tendonitis or tenosynovitis of the hand or forearm, occupational cancer, disease from occupational exposure to a biological agent;
   - dangerous occurrences, e.g. explosion of a pressure vessel, an electrical fire, or a staff sharps injury from a sharp **known** to be contaminated with a blood-borne virus. If the worker later sero-converts, that is reported as a disease. A sharps injury where the source is not BBV-positive or cannot be traced is not reportable unless it causes an over-seven-day injury or a disease (Policy 29 – Occupational Exposure Management (including Sharps) Policy).
5. **Over-three-day incapacitation** (not counting the day of the accident, but including weekends and rest days) is not reported, but must be recorded.
6. **How to report:** online via the HSE Incident Contact Centre form. The telephone line (0345 300 9923) is for deaths and major injuries only. Deaths, major injuries and non-worker injuries needing hospital treatment are reported without delay; other reports within the HSE timescales [check current HSE guidance for the deadline for over-seven-day injuries, diseases and dangerous occurrences].
7. The HSE sends back a copy of the report. File it with the accident book record.
8. Every incident, reportable or not, is investigated and, where needed, procedures are changed (significant event analysis – Policy 37 – Risk Management and Significant Events Policy).

6. Records and monitoring

Record	Where	Kept / reviewed
Health and safety risk assessment (incl. stress), individual assessments	H&S file [location / shared drive]	Reviewed at least annually and after changes
Accident book (BI510) and RIDDOR reports	Locked cabinet [location]	At least 3 years

Record	Where	Kept / reviewed
Over-3-day absence records	With accident book	At least 3 years
PAT test certificates, formal visual inspection log, user fault log	H&S file	Per test interval
Fixed wire test certificate	H&S file	Until next test
Written schemes of examination, pressure vessel inspection certificates, insurance	Pressure systems file	Current plus previous
DSE assessments	H&S file	On change of user or workstation
Manual handling assessments	H&S file	Annually
Medical device incident reports, lab MHRA registration, patient statements	SEA file / Dentally patient record	As per Policy 36 – Clinical Record-keeping and Retention Policy
Laser file (if applicable): LPA appointment, local rules, training, HIS registration	Laser file	Annually
Training records (all health and safety training)	Staff files	Kept for duration of employment [plus period]

The Health & Safety Lead completes the PSM H&S General Checklist each year and reports to a practice meeting.

7. Training

- Induction for every new starter: this policy, fire procedure, first aid arrangements, accident reporting, PPE, COSHH, manual handling, DSE (if a user). Recorded and signed.
- Manual handling training, including helping patients to the chair: on induction and [every 3 years – practice decision].
- DSE training for users and assessors.
- Equipment training for sterilizers, compressor and any laser before use.
- Formal visual inspection training for the in-house competent person.
- Health and safety training is a legal requirement; all training is recorded.

8. Related policies and templates

PSM templates to download and adapt:

- Health & Safety Policy (Oct 2025)
- Health & Safety Risk Assessment (Oct 2025)
- Display Screen Equipment Assessment (Oct 2025); Healthy Working Lives DSE workstation assessment
- Manual Handling Information (Oct 2025); Manual Handling Risk Assessment (Healthy Working Lives)
- H&S General Checklist (blank and worked example, Mar 2021)
- GDC Statement of Manufacture template

Related policies in this set: Policy 18 – COSHH Policy (Hazardous Substances, Mercury and Amalgam), Policy 19 – Fire Safety Policy, Policy 20 – First Aid Policy, Policy 21 – Latex Allergy and Contact Dermatitis Policy, Policy 22 – Waste Management Policy, Policy 23 – Personal Protective Equipment (PPE) Policy, Policy 24 – Infection

Prevention and Control Policy, Policy 26 – Decontamination of Reusable Instruments Policy and Procedures, Policy 29 – Occupational Exposure Management (including Sharps) Policy, Policy 31 – Dispatch to Laboratories, Equipment Suppliers and Pathology Services Policy, Policy 34 – Medical Emergencies and Life Support Policy, Policy 35 – Radiation Protection Policy, Policy 37 – Risk Management and Significant Events Policy, Policy 07 – Raising Concerns and Whistleblowing Policy, Policy 09 – Disability Equality and Reasonable Adjustments Policy.

9. Inspection checklist

- ☐ Written, signed and dated health and safety policy, shared with the team
- ☐ Recorded health and safety risk assessment (including stress), with individual assessments where needed
- ☐ Competent person for health and safety appointed
- ☐ HSE Health and Safety Law poster displayed (or leaflets issued)
- ☐ Current employer's liability insurance certificate displayed
- ☐ Data protection compliant accident book; RIDDOR procedure known to staff
- ☐ PAT and formal visual inspection records; fixed wire test certificate
- ☐ Written scheme of examination and in-date inspection certificate for each sterilizer and compressor; pressure vessel insurance
- ☐ DSE assessments for users; manual handling assessment and training
- ☐ Health and safety training records for all staff
- ☐ Laboratory MHRA registration checked; patient statements issued for custom-made devices
- ☐ If a laser is used: LPA appointed, local rules, controlled area, signs, eyewear, HIS registration
- ☐ Welfare facilities: staff rest area away from clinical areas, drinking water, secure storage

Staff sign-off

Name	Role	Signature	Date